

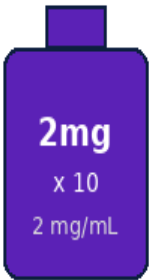
CAGRILINTIDE

Long-Acting Amylin Analogue (Amylin Receptor Agonist)
Complete Injection & Dosing Guide -- All Vial Sizes

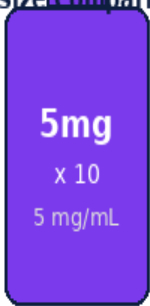
Purity: 99.8% | Endotoxin-Free | Heavy Metal-Free | Research Use Only

Vial Size Comparison

Cagrilintide Vial Size Comparison -- SaiyanMed



2mg
1mL BAC
2 mg/mL



5mg
1mL BAC
5 mg/mL



10mg
2mL BAC
5 mg/mL

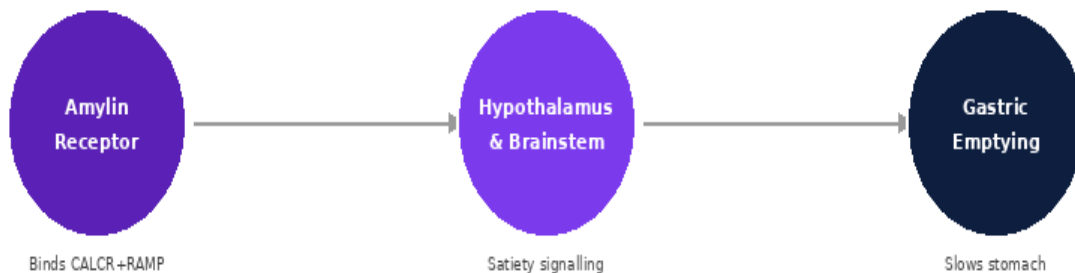
Vial	BAC Water	Concentration	Doses (0.3mg ea)	Examples per Kit (x10)
2mg x 10	1 mL	2.0 mg/mL	6	6 doses (0.3mg ea) 3 doses (0.6mg ea) 1 dose (2mg)
5mg x 10	1 mL	5.0 mg/mL	16	16 doses (0.3mg ea) 8 doses (0.6mg ea) 4 doses (1.2mg ea)
10mg x 10	2 mL	5.0 mg/mL	33	33 doses (0.3mg ea) 16 doses (0.6mg ea) 8 doses (1.2mg ea)

WHAT IS CAGRILINTIDE?

Cagrilintide is a **long-acting amylin analogue** designed to mimic the actions of amylin — a pancreatic hormone co-secreted with insulin that plays a critical role in post-meal satiety, gastric emptying regulation, and glucose homeostasis. Developed by Novo Nordisk, Cagrilintide acts on amylin receptors (CALCR + RAMP1/3) in the hypothalamus and brainstem, producing robust appetite suppression and body weight reduction.

In the landmark **CALM Phase 2 trial**, Cagrilintide 4.5mg alone achieved ~10.8% weight reduction over 26 weeks. In combination with semaglutide 2.4mg (the CagriSema combination, currently in Phase 3 REDEFINE trials), weight reduction exceeded 15% — with the REDEFINE-1 Phase 3 data showing up to 22.7% body weight reduction at 68 weeks.

Cagrilintide Mechanism -- Amylin Receptor Pathway



Receptor / Pathway	Primary Action	Research Benefit
Amylin Receptor (CALCR + RAMP1/3)	Binds calcitonin receptor complexes in hypothalamus and area postrema; activates central satiety circuits	Potent appetite suppression independent of GLP-1 pathway; additive effect when combined with GLP-1 agonists
Gastric Motility	Slows gastric emptying; reduces post-meal glucose excursion; prolongs satiety after meals	Reduced caloric intake; blunted post-prandial glucose spikes; improved glycaemic control
Glucagon Suppression	Post-prandial glucagon suppression; reduces hepatic glucose output	Complementary glycaemic benefit; particularly relevant in Type 2 diabetes research models
Body Weight Centres	Acts on hypothalamic POMC neurons and area postrema to reduce food intake and increase energy expenditure	Durable weight reduction; distinct from and complementary to GLP-1 receptor agonist mechanisms

Key Research Benefits

Benefit	Evidence & Research Context
Robust Weight Reduction	CALM Phase 2: 4.5mg monotherapy achieved ~10.8% weight loss over 26 weeks. REDEFINE Phase 3 (CagriSema): up to 22.7% at 68 weeks — comparable to surgical outcomes.
Complementary to GLP-1 Agonists	Amylin and GLP-1 pathways are distinct and additive. CagriSema (Cagrilintide + Semaglutide) outperforms either agent alone across all weight reduction metrics.
Durable Appetite Suppression	Once-weekly dosing with ~7-day half-life maintains consistent receptor engagement without peak-trough fluctuations associated with shorter-acting agents.
Glycaemic Control	Post-prandial glucose suppression via gastric emptying delay and glucagon inhibition; relevant for Type 2 diabetes and metabolic syndrome research.
Tolerability & Lean Mass	GI side effects are manageable with slow escalation; emerging data suggests preferential fat mass reduction with lean mass preservation vs caloric restriction alone.

RECONSTITUTION PROTOCOL | DOSE CALCULATION

Always use **Bacteriostatic Water (BAC Water)** for reconstitution. Allow both the Cagrilintide vial and BAC Water to reach room temperature before mixing. Use a fresh insulin syringe for each reconstitution.

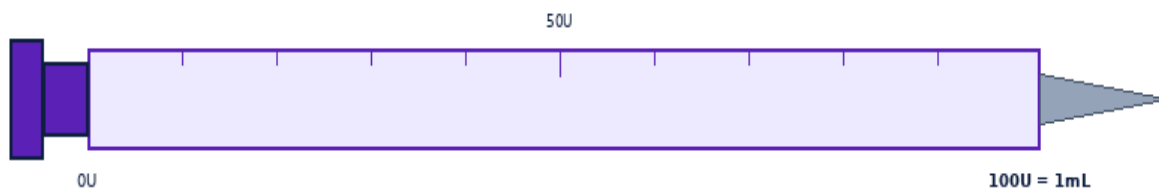


Step	Action	Detail
1	Gather supplies	Cagrilintide vial, BAC Water vial, insulin syringes (29-31G), alcohol swabs, sharps container, date labels.
2	Clean vial stoppers	Wipe rubber stoppers of BOTH vials with a fresh alcohol swab. Air-dry for 10-15 seconds before proceeding.
3	Draw BAC Water	Draw the correct volume per vial size (see Page 1 table). Expel all air bubbles before injection.
4	Inject into peptide vial	Direct stream against the GLASS WALL — not onto the powder. Inject slowly and steadily over 10-15 seconds.
5	Mix gently	Roll vial gently between palms for 20-30 seconds. NEVER shake — agitation degrades the peptide.
6	Inspect solution	Must be clear and colourless. Discard immediately if cloudy, discoloured, or contains visible particles.
7	Label and refrigerate	Write reconstitution date on vial. Store at 2-8 deg C. Use within 28 days. Never freeze.

Reconstitution Quick Reference

Vial Size	BAC Water	Resulting Concentration	Storage	Use Within
2mg	1 mL	2 mg/mL	2-8 deg C refrigerated	28 days
5mg	1 mL	5 mg/mL	2-8 deg C refrigerated	28 days
10mg	2 mL	5 mg/mL	2-8 deg C refrigerated	28 days

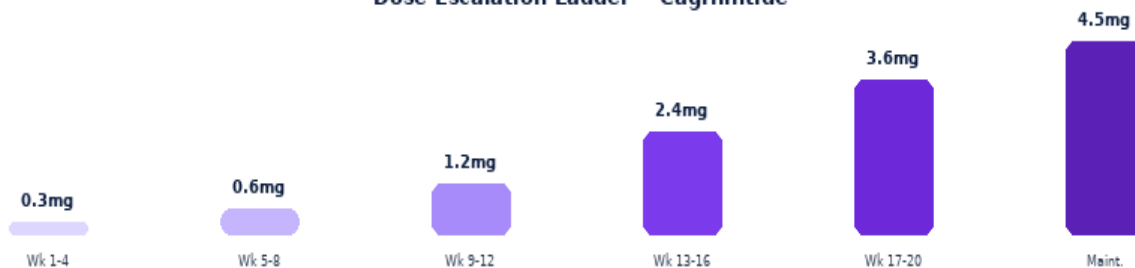
DOSE CALCULATION GUIDE



Insulin Syringe -- 1mL / 100 Units (29-31G needle)

Formula: Volume (mL) = Desired Dose (mg) / Concentration (mg/mL) | **Syringe Units:** Volume (mL) x 100 = Units on insulin syringe

Dose Escalation Ladder -- Cagrilintide



Vial: 2mg x 10 — Add 1 mL BAC Water → Concentration: 2.0 mg/mL

Dose (mg)	0.3mg	0.6mg	1.2mg	2.4mg	3.6mg	4.5mg
Volume (mL)	0.150	0.300	0.600	1.200	1.800	2.250
Syringe Units	15.0U	30.0U	60.0U	120.0U	180.0U	225.0U

Vial: 5mg x 10 — Add 1 mL BAC Water → Concentration: 5.0 mg/mL

Dose (mg)	0.3mg	0.6mg	1.2mg	2.4mg	3.6mg	4.5mg
Volume (mL)	0.060	0.120	0.240	0.480	0.720	0.900
Syringe Units	6.0U	12.0U	24.0U	48.0U	72.0U	90.0U

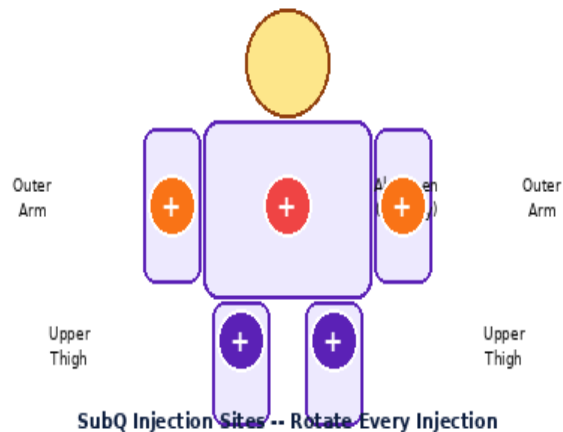
Vial: 10mg x 10 — Add 2 mL BAC Water → Concentration: 5.0 mg/mL

Dose (mg)	0.3mg	0.6mg	1.2mg	2.4mg	3.6mg	4.5mg
Volume (mL)	0.060	0.120	0.240	0.480	0.720	0.900
Syringe Units	6.0U	12.0U	24.0U	48.0U	72.0U	90.0U

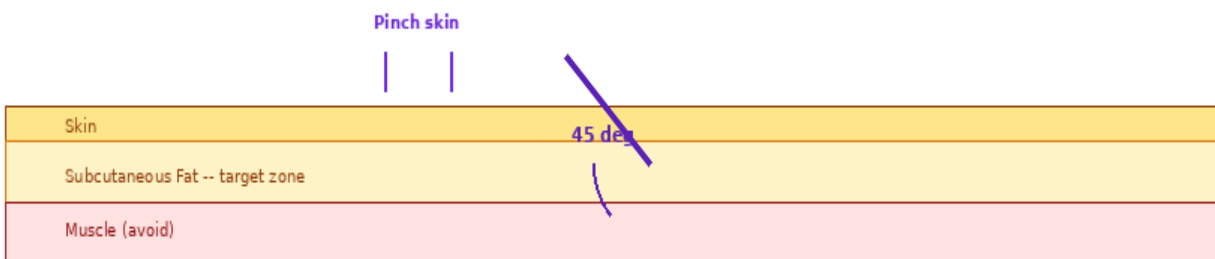
Important: Cagrilintide doses above 4.5 mg exceed Phase 3 trial doses. Always escalate in 0.3 mg increments every 4 weeks. Never exceed one injection per week.

INJECTION TECHNIQUE

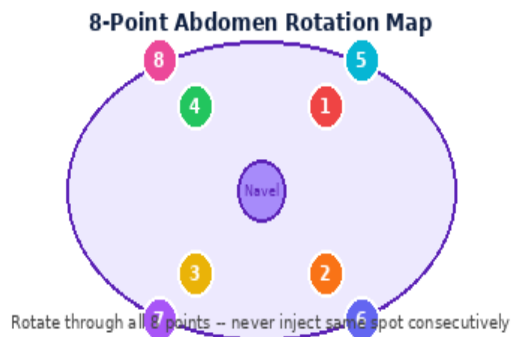
Step	Action	Detail
1	Wash hands	Wash with soap and warm water for 20 seconds. Dry completely.
2	Gather materials	Insulin syringe (29-31G x 5/16"), Cagrilintide vial, alcohol swabs, sharps container.
3	Clean injection site	Wipe chosen site in a circular motion with an alcohol swab. Allow 10-15 seconds to dry completely before injecting.
4	Draw the dose	Draw the correct volume per dose table. Tap syringe and expel all air bubbles before proceeding.
5	Pinch and insert	Pinch 2-3 cm of skin. Insert needle at 45 degrees. Release pinch. Inject slowly over 5-10 seconds.
6	Withdraw and press	Remove at same 45-degree angle. Press site gently 5-10 sec with dry swab. Do NOT rub — rubbing displaces the dose.
7	Safe disposal	Place needle immediately into sharps container. Never recap, reuse, or leave needles exposed.



45-Degree SubQ Injection Angle



SITE ROTATION & DOSING PROTOCOL



Weekly Dosing Schedule -- Same Day Each Week



Choose any consistent day (Mon shown). 1 injection per week only.

Phase	Weeks	Dose	Frequency	Notes
Starter	1-4	0.3 mg	1x / week	Assess GI tolerance. Nausea is common — inject with food.
Step 2	5-8	0.6 mg	1x / week	Increase only if 0.3 mg is well tolerated for 4 weeks.
Step 3	9-12	1.2 mg	1x / week	First clinically significant dose. Monitor appetite and weight.
Step 4	13-16	2.4 mg	1x / week	Strong appetite suppression expected. Track caloric intake.
Step 5	17-20	3.6 mg	1x / week	Escalate only if prior dose tolerated for full 4 weeks.
Maintenance	20+	4.5 mg	1x / week	Maximum Phase 3 trial dose. Titrate down if side effects persist.
Rest Period	Post-cycle	0 mg	4-8 wk off	Allow receptor sensitivity to reset before repeating cycle.

Administration tip: Inject Cagrilintide with or shortly after a meal — nausea is more pronounced when injected in a fasted state, especially during the early escalation phase.

CagriSema stacking note: When combining with Semaglutide, inject each compound separately at different sites on the same day. Do not mix in the same syringe.

SIDE EFFECTS, STORAGE & GOLDEN RULES

Side Effect	Likelihood	Management
Nausea	Very Common	Inject with food. Small, low-fat meals. Avoid strong smells on injection day.
Reduced Appetite	Very Common	Expected therapeutic effect. Maintain adequate protein and micronutrient intake.
Vomiting	Common	Usually dose-related. Hold escalation until resolved. Stay hydrated.
Diarrhoea	Common	Typically transient. Stay hydrated. Reduce dose if persistent.
Constipation	Occasional	Increase fluid and dietary fibre intake. Usually self-resolving.
Injection site reaction	Occasional	Rotate sites systematically. Use fresh 29-31G needle each time.
Hypoglycaemia (if on insulin)	Rare	Monitor blood glucose when combining with insulin. Reduce insulin dose as needed.
Tachycardia / Palpitations	Rare	Stop dosing. Seek medical advice before resuming.

STOP & SEEK MEDICAL ADVICE: Severe persistent abdominal pain (rule out pancreatitis)

STOP & SEEK MEDICAL ADVICE: Neck lump, hoarseness, or difficulty swallowing (thyroid concern)

STOP & SEEK MEDICAL ADVICE: Severe allergic reaction: hives, facial swelling, breathing difficulty

STOP & SEEK MEDICAL ADVICE: Resting heart rate increase > 20 bpm above personal baseline

STOP & SEEK MEDICAL ADVICE: Persistent severe vomiting or inability to keep fluids down

STOP & SEEK MEDICAL ADVICE: Jaundice or dark urine (hepatic concern)

Absolute Contraindications: Personal or family history of medullary thyroid carcinoma or MEN2 syndrome, active or history of pancreatitis, pregnancy or lactation. Not for use in individuals under 18.

Storage Guidelines

State	Temperature	Duration	Notes
Unreconstituted powder	2-8 deg C	Per expiry	Original vial. Dry, away from light and heat.
Reconstituted solution	2-8 deg C	28 days	Label with reconstitution date. Inspect before each use.
BAC Water (opened)	2-8 deg C	28 days	Label with opening date.
Short-term transport	Below 25 deg C	Max 72 hr	Insulated bag + ice packs. No direct ice contact.

Golden Rules — Cagrilintide

1. Always start at 0.3 mg and escalate in 0.3 mg steps every 4 weeks — no skipping.
2. Inject with or after food — nausea is markedly worse when injected in a fasted state.
3. One injection per week only. Maximum dose is 4.5 mg per week in research protocols.
4. Never inject the same site twice in a row — rotate through all 8 marked abdominal points.
5. Use only BAC Water for reconstitution. Never use saline or plain sterile water.

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| 6. Swirl gently to mix — NEVER shake. Agitation denatures the peptide. |
| 7. Label every vial with the reconstitution date. Discard after 28 days. |
| 8. Store at 2-8 deg C at all times. Never freeze reconstituted peptide. |
| 9. Contraindicated in thyroid cancer history, MEN2, pancreatitis, or pregnancy. |
| 10. If combining with Semaglutide (CagriSema), inject each compound separately at different sites. |

DISCLAIMER: This document is produced by [redacted] for informational and research reference purposes only. Cagrilintide is an investigational research compound. This guide does not constitute medical advice and must not be used to guide self-administration or human therapeutic use. Consult a qualified medical professional before beginning any peptide research protocol. [redacted] assumes no liability for misuse of this product or information contained herein.